

Depression & Anxiety — Outpatient Clinical Reference

Unipolar major depression & generalized anxiety disorder (GAD): screen → diagnose → treat → monitor. Adapted from TeachIM.org (CC BY-NC 4.0); USPSTF 2023 verified. Educational use — clinician-reviewed.

Screening tools

Tool	Role
PHQ-2 / PHQ-4	Brief screen (general population)
PHQ-9	Diagnose + monitor depression
GAD-7	Diagnose + monitor anxiety
Edinburgh / GDS-5	Perinatal / geriatric

Don't use PHQ-2/PHQ-4 when pretest probability is high → go straight to PHQ-9/GAD-7. Screening ~doubles detection (Mitchell, Lancet 2009).

PHQ-9 bands: 5–9 mild · 10–14 mod · 15–19 mod-severe · ≥20 severe (≥10 likely MDD).

Diagnose (DSM-5)

MDD: ≥5/9 sx ≥2 wk, nearly daily; **MUST** include depressed mood or anhedonia. SIG-E-CAPS: Sleep, Interest, Guilt, Energy, Concentration, Appetite, Psychomotor, Suicidality.

GAD: excessive, hard-to-control worry >half the days ×≥6 mo + ≥3 of: restlessness, fatigue, poor concentration, irritability, muscle tension, sleep disturbance.

Both: functional impairment; exclude substance/medical cause.

Before you treat

Screen for bipolar (antidepressant can unmask mania): age <25 · family hx bipolar · ≥3–5 MDD recurrences.

Labs to consider (none mandatory): CBC (anemia), TSH (thyroid), HCG (pregnancy + drug safety), BMP (renal dosing). Screen substance use (AUDIT/CAGE).

Mimics: adjustment d/o, ADHD, borderline PD, complicated grief, delirium, hypothyroidism, schizophrenia/schizoaffective, SUD; (anxiety) panic, OCD, PTSD, separation anxiety.

Treat & monitor

SRI **and/or** psychotherapy — comparable response, additive. Set target (remission = absence of sad mood + anhedonia ~3 wk).

Visit	Check → action
1–2 wk	No worsening SI/mania, no intolerable SE → if intolerable, switch agent
2–4 wk	Response? none → switch; partial → ↑dose, recheck 2wk; complete → maintain (d/c at 3–9 mo)

Stop by **slow taper** — withdrawal can be prolonged. ANTLER (NEJM 2021): ~56% relapse off vs ~39% maintained.

Pick the agent by side effect

Want / feature	Reach for
Wt gain + sedation (poor appetite, insomnia, elderly)	Mirtazapine
Polypharmacy (few interactions)	Citalopram/escitalopram (QTc; max 40, 20 elderly)
More activation	Sertraline, fluoxetine, bupropion
Comorbid chronic pain	SNRI (duloxetine, venlafaxine)
Avoid wt gain / withdrawal	Avoid paroxetine; fluoxetine = longest t½

"Start low, go slow," esp. elderly. SRIs first-line for BOTH disorders.

Safety assessment (every visit)

Use **clear language**: "Are you having thoughts of killing yourself right now?" Asking does **NOT** increase risk. Tools: PHQ-9 item 9, Columbia (C-SSRS). Know your site's safety plan + immediate MH resources.

2022–25 updates

- **USPSTF 2023:** screen ALL adults for depression (B) & adults <65 for anxiety (B), incl. pregnant/postpartum; ≥65 anxiety = I.
- **Auvelity** (dextromethorphan–bupropion, 2022): rapid-acting oral NMDA antagonist for MDD.
- **Zuranolone** (Zurzuvae, 2023): first oral agent for postpartum depression (14-day course).
- Esketamine (Spravato): treatment-resistant MDD; monotherapy 2025.

Red flags → act

- Active SI/plan/intent → safety plan, urgent MH eval.
- Post-start surge in energy + ↓sleep need → reassess for (hypo)mania/bipolar.
- Emergent suicidality/mania in first weeks → stop/change, escalate.